ISSUED, VALID FROM 30 SEPTEMBER 2026

Acute Otitis Externa

Patient Group Direction, version 008, issued 30 September 2026. Two products: ciprofloxacin 2mg/ml single-dose ear drops, and dexamethasone with neomycin and acetic acid ear spray.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this document covers, and what changed

- **THIS IS AN OTITIS EXTERNA SERVICE. It is not acute otitis media and it does not authorise any oral antibiotic. A patient with otitis media is referred.**
- **CIPROFLOXACIN 2MG/ML EAR DROPS ARE NOW AUTHORISED BY THIS DOCUMENT.**
- **Otoscopy is a required step and a required competency. The tympanic membrane must be seen and recorded as intact before either product is supplied.**
- **The red flags that sat in the guidance summary are now exclusion criteria in both arms.**
- **ONE COURSE PER EPISODE. A patient who has not improved after a full course is referred, not re-supplied.**

Choosing between the two products

- **Where the tympanic membrane is intact and clearly seen, either product may be used. Ciprofloxacin drops are preferred where there is any doubt about the drum, because neomycin is an aminoglycoside and is potentially ototoxic if it reaches the middle ear.**
- **Ciprofloxacin drops are licensed from 1 year of age. The spray is licensed from 2 years. For a child aged 1 to under 2, only the drops may be used.**
- **Ciprofloxacin drops may be used in pregnancy and while breastfeeding, because systemic exposure after otic administration is negligible. The spray is not recommended in pregnancy. In a pregnant or breastfeeding woman, use the drops.**
- The spray contains a corticosteroid and may settle a very inflamed, itchy canal faster. That is its advantage, and the only reason to choose it over the drops.
- Do not use both. Do not use any other ear preparation at the same time.

General measures for every patient

- Keep water out of the ear for the whole course and for a week afterwards. No swimming. Use a shower cap or petroleum jelly on cotton wool.
- No cotton buds, no earplugs, no self-cleaning. This is usually what started it.
- Paracetamol or ibuprofen for pain, as a separate pharmacy sale subject to the usual checks. Pain relief matters: otitis externa hurts.
- **Where discharge or debris is obstructing the canal so drops cannot reach the skin, refer for aural toilet. Do not attempt to clear the canal in the pharmacy.**

Ciprofloxacin 2mg/ml ear drops, single-dose containers

Summary of the Otomize and Cetraxal SmPCs and BNF and NICE CKS guidance for otitis externa

Ennogen Healthcare International Ltd, Otomize Ear Spray (dexamethasone 0.1 percent, neomycin sulfate 0.5 percent, glacial acetic acid 2 percent w/w) Summary of Product Characteristics, text revised 23 December 2024, emc updated 10 July 2025. Aspire Pharma Ltd, Cetraxal 2 mg/ml ear drops solution in single-dose container (ciprofloxacin) Summary of Product Characteristics, text revised 31 October 2017, emc updated 3 January 2018. BNF treatment summaries Ear and Ear infections, antibacterial therapy, bnf.nice.org.uk, read 10 September 2026. NICE Clinical Knowledge Summaries, Otitis externa, last revised August 2025 (definition, diagnosis, assessment, differential diagnosis and acute otitis externa scenario). Summarised 10 September 2026.

What otitis externa is

CKS defines otitis externa as diffuse inflammation of the skin and subdermis of the external ear canal, which may also involve the pinna or tympanic membrane; it is also known as swimmer's ear or tropical ear. A UK consensus document defines acute otitis externa as inflammation of less than 6 weeks' duration, typically caused by bacterial infection with Pseudomonas aeruginosa or Staphylococcus aureus. Chronic otitis externa has lasted longer than 3 months and persistent inflammation may be fungal (Aspergillus species or Candida albicans).

The BNF describes it as inflammation of the external ear canal which in some cases may involve oedema, primarily caused by bacterial infection, and states it is important to consider underlying otitis media as otitis externa may be secondary to otorrhoea from otitis media.

CKS: suspect acute otitis externa with at least one typical symptom, usually of rapid onset within 48 hours (itch of the ear canal; ear pain and tenderness of the tragus and/or pinna, often severe, with possible jaw pain; ear discharge; less commonly hearing loss from canal occlusion) and at least two typical signs (tenderness of the tragus and/or pinna; a red, oedematous ear canal, possibly with debris and discharge causing swelling and occlusion; tympanic membrane erythema; cellulitis of the pinna and adjacent skin; less commonly conductive hearing loss or tender regional lymphadenitis).

CKS: chronic otitis externa presents with constant itch, rarely mild discomfort, a lack of ear wax, dry scaly or red moist skin in the canal with partial stenosis, fluffy cotton-like debris, hyphae or black dots if fungal, and conductive hearing loss.

Distinguishing it from otitis media and from malignant otitis externa

CKS differential diagnosis: acute otitis media may present with an erythematous tympanic membrane and ear discharge, particularly if a tympanostomy tube is in situ. Other mimics are a foreign body (especially in children, with purulent discharge and pain), impacted ear wax, skin conditions involving the canal (contact dermatitis, eczema, fungal infection, psoriasis, seborrhoeic dermatitis, erysipelas, discoid lupus), referred pain from teeth, sphenoidal sinus, neck or throat, cholesteatoma (persistent or recurrent discharge and fullness, typically painless, with possible perforation, retraction pocket and granulation tissue that may mimic malignant otitis externa), mastoiditis (systemic illness, fever, marked hearing loss, mastoid tenderness or swelling), Ramsay Hunt syndrome (severe pain, vesicles on the canal and posterior pinna, facial paralysis), barotrauma, and squamous cell carcinoma of the canal, which can present like malignant otitis externa with abnormal tissue growth or bloody discharge.

CKS defines malignant (necrotising) otitis externa as a potentially life-threatening progressive infection of the external ear canal which may spread to cause osteomyelitis of the temporal bone and adjacent structures, commonly caused by Pseudomonas aeruginosa.

CKS: suspect malignant otitis externa with unremitting, disproportionate ear pain, headache, purulent otorrhoea, fever or malaise, vertigo, or profound conductive hearing loss, and signs of being systemically unwell with high fever, granulation tissue on the floor of the ear canal at the bone-cartilage junction or exposed bone in the canal, and ipsilateral facial nerve palsy.

CKS assessment: ask about onset, nature and severity, discharge, itch, fever, hearing loss, recent ear trauma, hearing aids or ear plugs, head or neck radiotherapy, previous episodes and treatments, previous ear surgery, perforation of the tympanic membrane or tympanostomy tube within the previous year, contact dermatitis, and diabetes or other immunocompromise. Examine the unaffected or less affected ear first; assess the canal, tympanic membrane, pinna and lymph nodes; look for signs of fungal infection; assess for a perforation or tympanostomy tube; note that the tympanic membrane can be difficult to visualise at first presentation.

The BNF states that in view of reports of ototoxicity, treatment with topical aminoglycosides is contraindicated in patients with a perforated tympanic membrane; cautious use in the presence of a perforation or patent grommet is a specialist decision only, for no longer than 2 weeks, with counselling on ototoxicity and baseline audiometry where possible.

How the BNF and CKS frame treatment

BNF: any precipitating factors should be managed and cleaning the canal considered (dry swabbing, microsuction or irrigation) if wax or debris block passage of topical medicine, which may need specialist referral. Acetic acid 2 percent acts as an astringent, may be used to treat mild otitis externa and is comparable to an anti-infective combined with a corticosteroid, with efficacy reduced if treatment extends beyond 1 week.

BNF: if infection is present, a topical anti-infective with or without a corticosteroid may be considered. These are used for a minimum of one week, but if symptoms persist can be used until they resolve, up to a maximum of 2 weeks. Prolonged and extensive use may affect the flora of the canal and increase the risk of fungal infection, and sensitivity to topical ear preparations may occur, especially with prolonged or recurrent use.

BNF antibacterial choice: ciprofloxacin (or an aminoglycoside) if pseudomonas is suspected; oral flucloxacillin, or clarithromycin (or azithromycin or erythromycin) in penicillin allergy, where an oral antibacterial is needed. Oral antibacterials are rarely indicated and specialist advice should be considered if they are required.
BNF: for severe pain a simple analgesic such as paracetamol or ibuprofen is usually sufficient; codeine may be used for severe pain.
CKS: consider prescribing a topical antibiotic preparation with or without a topical corticosteroid for 7 to 14 days, depending on clinical judgement and symptom response. Choice should be guided by personal preference, risk of adverse effects including ototoxicity, cost, dosing frequency, and whether the tympanic membrane is intact (if visualised). An additional topical corticosteroid may be beneficial where there is significant inflammation, erythema and oedema of the canal. Advise on how to administer ear drops correctly.
CKS: consider aural toilet (dry swabbing of secretions, or irrigation if the tympanic membrane is visualised and intact and the person is not immunocompromised) so that topical treatment can be applied effectively. Consider an ear swab for bacterial and fungal culture in treatment failure, severe, recurrent or chronic disease, canal occlusion, or suspected spread beyond the canal.

Otomize ear spray: the SmPC

Composition: neomycin sulfate 0.5 percent w/w (3250 IU/ml), dexamethasone 0.1 percent w/w and glacial acetic acid 2 percent w/w, as a milky oil-in-water emulsion sprayed into the external auditory meatus. Neomycin is a broad-spectrum antibiotic, dexamethasone a topical anti-inflammatory steroid, and the acetic acid produces a low pH to assist control of bacterial infection. It acts locally.
Indication: the treatment of otitis externa.
Dose for adults (including the elderly) and children 2 years of age and over: one metered dose (60 mg) administered directly into each affected ear three times daily. Treatment should be continued until two days after symptoms have disappeared.
The course rule: discontinue treatment if there is no clinical improvement after 7 days. Corticosteroid and antibiotic combinations should not be continued for more than 7 days in the absence of clinical improvement, because prolonged use may lead to occult extension of infection due to the masking effect of the steroid. Product use should be discontinued, and medical advice sought where appropriate, if irritation or rash occurs, or if the condition worsens or does not improve within 7 days.
Method: shake well before use; before first use press the actuator down several times to obtain a fine spray, and re-prime if more than a week has passed since last use; place the nozzle tip gently into the ear opening and press once; do not inhale the spray; use within one month of first use. Store upright in the carton, not above 25 degrees C, do not freeze.
Contraindications: hypersensitivity to the active substances or any excipient; patients in whom a perforated tympanic membrane has been diagnosed or is suspected, or where a tympanostomy tube (grommet) is in situ; infants and neonates under 2 years of age.
The perforation warning: when otitis externa is treated topically with aminoglycoside-containing preparations in patients with a perforated tympanic membrane there is an increased risk of drug-induced deafness, so it is important to ensure there is no perforation. It is also important to exclude chronic alternative diagnoses, including chronic otitis media, before treatment is commenced.
Other cautions: aminoglycosides may cause irreversible partial or total deafness when applied topically to open wounds or damaged skin; the effect is dose related and enhanced by renal or hepatic impairment, and should be considered with high doses or prolonged treatment in small children. Increased ototoxicity has been observed with aminoglycosides in patients with mitochondrial mutations, particularly m.1555A>G, sometimes with a maternal history of deafness; no cases were identified with neomycin but a similar effect is possible. In children there is a theoretical risk of adrenal suppression with prolonged use, and neomycin toxicity may develop because of immature renal function. Prolonged use may lead to skin sensitisation and resistant organisms. Blurred vision or other visual disturbance should prompt consideration of ophthalmology referral. The product contains methyl and propyl hydroxybenzoates, which may cause delayed allergic reactions, and stearyl alcohol, which may cause contact dermatitis.
Pregnancy: not recommended; there are no adequate data on dexamethasone in pregnancy and a risk of fetal ototoxicity with aminoglycosides. Lactation: a risk to the breastfed child cannot be excluded and a decision must be made whether to discontinue breastfeeding or the treatment, although topical use generally carries less risk than systemic.
Adverse effects: some patients experience a transient stinging or burning sensation for the first few days; skin sensitisation and immediate or delayed hypersensitivity reactions causing irritation, burning, stinging, itching and dermatitis; blurred vision with corticosteroid use. Unresolved ear problems could themselves affect driving ability. No interactions are known and overdose by this route is extremely unlikely.

Cetraxal ciprofloxacin 2 mg/ml ear drops: the SmPC

Composition: 1 ml contains ciprofloxacin 2 mg as hydrochloride; each single-dose ampoule delivers 0.25 ml containing 0.5 mg of ciprofloxacin, as a clear, sterile, preservative-free aqueous solution. Each pack contains 15 ampoules. It is a fluoroquinolone that inhibits bacterial DNA gyrase and topoisomerase IV.
Indication: the treatment of acute otitis externa in adults and children older than 1 year with an intact tympanic membrane, caused by ciprofloxacin-susceptible microorganisms. Consideration should be given to official guidance on the appropriate use of antibacterial agents.

Dose for adults and children aged one year and older: instil the contents of one single ampoule into the affected ear twice daily for seven days. Safety and efficacy below 1 year have not been established, and the prescriber should weigh benefits against known and unknown risks in that age group. No dose adjustment is needed in renal or hepatic impairment because plasma concentration is expected to be undetectable.
Method: warm the solution by holding the ampoule in the hand for several minutes to avoid the dizziness that a cold solution can cause; the patient lies with the affected ear upward, the drops are instilled while pulling several times on the auricle, and the position is held for around 5 minutes; repeat for the other ear if needed; discard the ampoule after use and do not keep it. If an otowick or tampon is used the first dose should be doubled (2 ampoules). Store below 30 degrees C in the original packaging to protect from light; the pouch is in date for 8 days after opening.
Contraindications: hypersensitivity to ciprofloxacin, to any member of the quinolone class, or to any excipient.
What the SmPC says about perforation: the safety and efficacy of Cetraxal have not been studied in the presence of a perforated tympanic membrane, so it should be used with caution in patients with known or suspected perforation, or where there is a risk of perforation.
Other cautions: for auricular use only, not ophthalmic, inhalation or injection; meticulous medical monitoring is required so that the need for other measures can be recognised in time; discontinue at the first appearance of a skin rash or any other sign of hypersensitivity, since serious and occasionally fatal anaphylactic reactions have been reported with systemic quinolones; use may result in overgrowth of non-susceptible organisms including yeasts and fungi, and superinfection needs appropriate therapy. Photoallergic reactions are unlikely at this site.
The course rule: if after one week of therapy some signs and symptoms persist, further evaluation is recommended to reassess the disease and the treatment.
Interactions: none studied; systemic interaction is unlikely because of low plasma levels, but it is recommended not to use other ear preparations concomitantly.
Pregnancy and breastfeeding: systemic exposure after otic administration is negligible and the SmPC states Cetraxal can be used during pregnancy and during breastfeeding.
Adverse effects, all uncommon (1 in 1,000 to 1 in 100): ear pruritus, tinnitus, dizziness, headache, dermatitis and application site pain. With locally applied fluoroquinolones, generalised rash, toxic epidermolysis, exfoliative dermatitis, Stevens-Johnson syndrome and urticaria occur very rarely. Overdose risk is negligible (7.5 mg ciprofloxacin per pack). No or negligible effect on driving.

When to refer or seek advice

CKS: arrange emergency hospital admission or urgent ENT referral if malignant otitis externa or a serious complication is suspected.
CKS: seek specialist advice or arrange ENT referral, urgency depending on clinical judgement, if symptoms persist despite optimal primary care management; there is severe infection not responding to primary care management; the person is elderly, has poorly controlled diabetes mellitus or another cause of immunocompromise; there is canal occlusion from discharge, swelling or debris that stops topical treatment working (specialist aural toilet, microsuction or an ear wick may be needed); or there is cellulitis extending beyond the canal which cannot be managed in primary care, where systemic antibiotics may be needed on specialist advice.
CKS: consider an oral antibiotic if the person is immunocompromised, there is severe infection, or infection has spread beyond the external ear canal to adjacent tissues. BNF: a systemic antibacterial may be considered if infection is spreading outside the canal, the patient is systemically unwell, or in a high-risk group such as diabetics, immunocompromised patients, or those with severe infection or at high risk of severe infection such as pseudomonal infection; referral should be considered if there is extensive swelling of the auditory canal.
CKS follow-up triggers: symptoms not improving within 48 to 72 hours of starting treatment; symptoms not fully resolved after 2 weeks; severe symptoms or spreading cellulitis; an immunocompromised person at risk of severe infection; or wax impaction or canal stenosis preventing the tympanic membrane being visualised, to check for an alternative cause.
SmPC stop points: Otomize should be discontinued if there is no clinical improvement after 7 days, or if irritation, rash or worsening occurs; Cetraxal needs further evaluation if signs and symptoms persist after one week, and should be stopped at the first sign of rash or hypersensitivity.
CKS treatment failure: reassess for alternative conditions and underlying risk factors, check self-care and correct use of the drops, consider switching between drop and spray, switch away from neomycin or another aminoglycoside if contact sensitivity is suspected, re-examine for occlusion, fungal infection, foreign body, tympanic membrane perforation or middle ear disease, and consider an ear swab. Consider dermatology referral for patch testing if contact sensitivity to neomycin, ear plugs, hearing aids or earrings is suspected.

What to tell the patient

CKS self-care: avoid damaging the ear canal; cotton buds or other objects should not be used to clean the ear canal, and troublesome wax should be removed safely.
Keep the ears clean and dry. Avoid swimming and water sports for at least 7 to 10 days during treatment; use ear plugs and/or a tight-fitting cap when swimming; keep shampoo, soap and water out of the ear when bathing and showering, for example with ear plugs or cotton wool with petroleum jelly; consider a hair dryer on the lowest heat setting to dry the canal after hair washing, bathing or swimming.

Use paracetamol or ibuprofen for pain if needed. The NHS leaflet on ear infections is a source of information. Anyone allergic or contact-sensitive to ear drops such as neomycin, ear plugs, hearing aids or earrings should avoid them or use alternatives.

Otomize: shake, prime, one spray into each affected ear three times a day, continue until two days after symptoms have gone, and stop and seek advice if there is no improvement after 7 days or if irritation or a rash develops. A transient stinging or burning sensation is expected in the first few days. Use within one month of opening.

Cetraxal: warm the ampoule in the hand first, lie with the affected ear upward, instil the whole ampoule while pulling the ear, stay in position for about 5 minutes, discard the ampoule, twice a day for seven days; do not use other ear preparations at the same time; stop and seek help at the first sign of a rash; return if symptoms persist after the week.

Seek urgent review for severe or worsening pain out of proportion to the appearance of the ear, high fever or feeling systemically unwell, spreading redness or swelling around the ear, facial weakness, vertigo or marked hearing loss.

Return if there is no improvement within 48 to 72 hours of starting treatment, or if symptoms have not fully settled after 2 weeks.

Patient Group Direction for the supply of ciprofloxacin 2mg/ml ear drops solution in single-dose containers for the treatment of acute otitis externa in adults and children older than 1 year with an intact tympanic membrane.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- ALL USERS MUST BE COMPETENT IN OTOSCOPY. This PGD cannot be used without examining the ear. The tympanic membrane must be visualised and recorded as intact before either product is supplied, and otitis media, perforation, a grommet, a foreign body and fungal debris must be distinguishable from acute otitis externa.
- Competence in otoscopy must be documented and signed off by the employing organisation before the practitioner works under this PGD. Self-declaration is not sufficient.
- All users must be familiar with the current SPC for the product supplied, the BNF and NICE CKS on otitis externa.
- All users must be able to recognise necrotising (malignant) otitis externa and know that it is an emergency ENT referral, not a treatment failure.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent, including consent in children and Gillick competence, and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of acute otitis externa in adults and children older than 1 year with an intact tympanic membrane, caused by ciprofloxacin susceptible micro-organisms, in accordance with the current SPC and NICE CKS.
Inclusion criteria	● Aged 12 months or over (the drops are licensed from the first birthday). There is no upper age limit. ● Clinical signs and symptoms of acute otitis externa: ear pain, itch, discharge, a swollen or erythematous canal, or tenderness of the tragus or pinna. ● **OTOSCOPY PERFORMED, the tympanic membrane visualised, and recorded as INTACT.** ● **No red flag in Appendix 1 present.** ● No previous course supplied for this episode. ● Valid informed consent obtained, from a person with parental responsibility where the patient is under 16 and not Gillick competent. ● No exclusion criterion present.
Exclusion criteria: red flags	**Refer, do not supply, where ANY of the following is present. These are exclusions, not points to consider.** ● **SEVERE UNREMITTING PAIN IN A PATIENT WITH DIABETES OR WHO IS IMMUNOCOMPROMISED. Suspect necrotising (malignant) otitis externa. This is a same-day emergency ENT referral. It is the one presentation in this document that kills.**

	• **Facial nerve palsy, or any weakness or drooping of the face. Same-day emergency ENT referral.** • **Pain, swelling, redness or tenderness over the mastoid, behind the ear, or an ear that is pushed forward. Suspect mastoiditis. Same-day emergency referral.** • **Systemic illness: fever, rigors, or the patient appearing unwell. Refer the same day.** • Spreading infection: cellulitis of the pinna, the face or the neck. • Vertigo, new hearing loss beyond simple canal blockage, tinnitus of new onset, or any other neurological symptom. • **Failed treatment after 2 weeks, or failure to improve after a full course under this PGD. Refer; do not supply a second course blind.** • Suspected foreign body in the canal. • **White or black fuzzy debris in the canal, suggesting fungal infection. Antibacterial drops will not treat it and may make it worse. Refer.**
Exclusion criteria: all other	**Refer, do not supply, where any of the following applies.** • Aged under 12 months. Safety and efficacy below 1 year of age have not been established. • **Tympanic membrane not visualised, perforated, or perforation suspected. If you cannot see the drum, you cannot use this PGD.** • Grommet or tympanostomy tube in situ. • **Known hypersensitivity to ciprofloxacin, to any quinolone antibacterial, or to any excipient.** • **Otitis media, or any infection requiring an oral antibiotic. This document does not authorise one.** • Chronic otitis externa, meaning symptoms lasting more than 3 months. Refer to ENT. • **Recurrent otitis externa: 3 or more episodes in the last 12 months. Refer to ENT for investigation.** • Already using another ear preparation. Concomitant ear preparations are not recommended. • Valid informed consent not obtained.
Cautions	**Discontinue at the first sign of a skin rash or any other sign of hypersensitivity. Serious and occasionally fatal anaphylactic reactions, some after a first dose, have been reported with systemic quinolones.** Warm the ampoule in the hand for a few minutes before use. Instilling cold solution into the ear causes dizziness. **Use may allow overgrowth of non-susceptible organisms including yeasts and fungi. If the patient returns worse with fuzzy white or black debris, that is a fungal superinfection: refer, do not repeat.** If signs and symptoms persist after one week of treatment, further evaluation is needed. Refer. **For auricular use only. Not for the eye, not for inhalation, not for injection.**
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where a red flag is present, refer the same day, and for suspected necrotising otitis externa, facial palsy or mastoiditis, arrange emergency ENT assessment rather than a routine GP appointment. Give pain relief advice and the water-avoidance advice regardless of whether anything is supplied. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Ciprofloxacin 2mg/ml ear drops solution in single-dose container (for example Cetraxal). Each 0.25mL ampoule delivers 0.5mg ciprofloxacin.
Legal category	POM.
Dose and frequency	The contents of one single-dose ampoule instilled into the affected ear twice daily for 7 days. Where an otowick or tampon is used to aid administration, the FIRST dose only is doubled, using two ampoules.
Quantity to be supplied	One pack of 15 single-dose ampoules. A 7 day course at twice daily uses 14 ampoules, with one spare. Supply for one ear unless both are affected and both were examined. No repeat supply under this PGD.
Maximum treatment period	7 days. One course per episode. A patient not improving at the end of the course is referred, not re-supplied.
Route and method of administration	● Warm the ampoule by holding it in the hand for several minutes. ● The patient lies with the affected ear upward. Instil the drops, then pull gently on the auricle several times. ● Stay in that position for about 5 minutes to let the drops penetrate. Repeat for the other ear if both are affected. ● **Discard the ampoule after use. Do not keep it for the next dose.**
Storage	Store below 30 degrees Celsius, in the original packaging to protect from light. Once the foil pouch is opened, the remaining ampoules must be used within 8 days.
Drug interactions	No interaction studies have been performed. Systemic absorption after otic use is negligible, so systemic interactions are unlikely. Do not use other ear preparations at the same time.
Pregnancy and breastfeeding	May be used in both. Systemic exposure after otic administration is negligible and no effects are anticipated (SPC section 4.6). This is the arm to use in a pregnant or breastfeeding woman.
Adverse effects	Uncommon: ear pruritus, tinnitus, dizziness, headache, dermatitis, application site pain. Very rare with locally applied fluoroquinolones: generalised rash, toxic epidermal necrolysis, exfoliative dermatitis, Stevens-Johnson syndrome, urticaria.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given, and from whom. Where the patient is a child, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. ● Patient name, address, date of birth, and the GP with whom they are registered. ● **THE OTOSCOPY FINDING, in terms: which ear, the state of the canal, and that the tympanic membrane was seen and was intact. Where the drum could not be visualised, that fact and what was done about it.** ● **That every red flag in Appendix 1 was asked about or looked for, and was absent.** ● **Diabetes and immunosuppression status, and the severity of pain.** ● How long symptoms have been present, and any treatment already tried.

	• **Any previous episode in the last 12 months, and any previous course supplied under this PGD.** • Which product was supplied and why. • Name, form, strength, dose, quantity and date of supply. • Batch number and expiry date of the product supplied. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Dispose of used ampoules and any unused product in accordance with local requirements. Advise the patient to return unused ampoules to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Warm the ampoule in your hand first. Cold drops make you dizzy. • Lie with the sore ear facing up, put the drops in, tug the ear gently a few times, and stay there for 5 minutes. • Twice a day for 7 days. Use a new ampoule each time and throw it away afterwards. • **Keep water out of the ear for the whole course and a week after. No swimming.** • Nothing goes in the ear: no cotton buds, no earplugs. • Paracetamol or ibuprofen for the pain if they suit you.
Follow-up and when to get help urgently	**Seek help the SAME DAY if any of these happen:** • **The pain becomes severe and does not let up, particularly if you have diabetes or a weakened immune system.** • **Any weakness or drooping of your face.** • **Swelling, redness or pain in the bone behind your ear, or your ear starts to stick out.** • You develop a temperature or feel generally unwell. • A rash, or any swelling of the lips, face or throat. Stop the drops. **Otherwise, if you are no better after finishing the 7 days, go to your GP. Do not start a second course.**

Dexamethasone with neomycin sulfate and acetic acid ear spray

Patient Group Direction for the supply of dexamethasone 0.1% w/w, neomycin sulfate 0.5% w/w and acetic acid 2% w/w ear spray for the treatment of mild to moderate acute otitis externa in adults and children aged 2 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- ALL USERS MUST BE COMPETENT IN OTOSCOPY. This PGD cannot be used without examining the ear. The tympanic membrane must be visualised and recorded as intact before either product is supplied, and otitis media, perforation, a grommet, a foreign body and fungal debris must be distinguishable from acute otitis externa.
- Competence in otoscopy must be documented and signed off by the employing organisation before the practitioner works under this PGD. Self-declaration is not sufficient.
- All users must be familiar with the current SPC for the product supplied, the BNF and NICE CKS on otitis externa.
- All users must be able to recognise necrotising (malignant) otitis externa and know that it is an emergency ENT referral, not a treatment failure.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent, including consent in children and Gillick competence, and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of mild to moderate acute otitis externa in adults and children aged 2 years and over with an intact tympanic membrane, in accordance with the current SPC and NICE CKS.
Inclusion criteria	● Aged 2 years and over. There is no upper age limit. ● Clinical signs and symptoms of mild to moderate acute otitis externa. ● **OTOSCOPY PERFORMED, the tympanic membrane visualised, and recorded as INTACT.** ● **No red flag in Appendix 1 present.** ● No previous course supplied for this episode. ● **Not pregnant. Use the ciprofloxacin arm instead.** ● Valid informed consent obtained, from a person with parental responsibility where the patient is under 16 and not Gillick competent. ● No exclusion criterion present.
Exclusion criteria: red flags	**Refer, do not supply, where ANY of the following is present. These are exclusions, not points to consider.** ● **SEVERE UNREMITTING PAIN IN A PATIENT WITH DIABETES OR WHO IS IMMUNOCOMPROMISED. Suspect necrotising (malignant) otitis externa. This is a same-day emergency ENT**

	referral. It is the one presentation in this document that kills. ● **Facial nerve palsy, or any weakness or drooping of the face. Same-day emergency ENT referral.** ● **Pain, swelling, redness or tenderness over the mastoid, behind the ear, or an ear that is pushed forward. Suspect mastoiditis. Same-day emergency referral.** ● **Systemic illness: fever, rigors, or the patient appearing unwell. Refer the same day.** ● Spreading infection: cellulitis of the pinna, the face or the neck. ● Vertigo, new hearing loss beyond simple canal blockage, tinnitus of new onset, or any other neurological symptom. ● **Failed treatment after 2 weeks, or failure to improve after a full course under this PGD. Refer; do not supply a second course blind.** ● Suspected foreign body in the canal. ● **White or black fuzzy debris in the canal, suggesting fungal infection. Antibacterial drops will not treat it and may make it worse. Refer.**
Exclusion criteria: all other	**Refer, do not supply, where any of the following applies.** ● Aged under 2 years. Use the ciprofloxacin arm from 1 year, or refer. ● **Tympanic membrane not visualised, perforated, or perforation suspected. Neomycin is an aminoglycoside and is potentially ototoxic if it reaches the middle ear. If you cannot see the drum, use the ciprofloxacin arm or refer.** ● **Grommet or tympanostomy tube in situ.** ● Known hypersensitivity to neomycin, to any aminoglycoside, to dexamethasone, to acetic acid or to any excipient. ● **Pregnancy. This product is not recommended in pregnancy; use the ciprofloxacin arm.** ● **Breastfeeding, unless a decision has been made and recorded about whether to continue breastfeeding or the treatment. The ciprofloxacin arm avoids the question.** ● **Otitis media, or any infection requiring an oral antibiotic. This document does not authorise one.** ● Chronic otitis externa, meaning symptoms lasting more than 3 months. Refer to ENT. ● **Recurrent otitis externa: 3 or more episodes in the last 12 months. Refer to ENT.** ● Already using another ear preparation. ● Valid informed consent not obtained.
Cautions	**Confirm the tympanic membrane is intact before use. This is the single most important check for this product.** **Avoid prolonged use, to reduce the risk of sensitisation to neomycin and of resistance. Neomycin sensitisation is common and lasts a lifetime.** Shake the bottle well before use and advise the patient to keep water out of the ear. Refer if there is no improvement within 7 days, if symptoms worsen, or if irritation or rash occurs. Visual disturbances have been reported with corticosteroid use. Refer to an ophthalmologist for evaluation where a patient reports blurred vision or other visual disturbance.
Actions if the patient is excluded or declines	As for the ciprofloxacin arm. Where the exclusion is pregnancy, breastfeeding, an uncertain drum or an age below 2 years, consider the ciprofloxacin arm

	rather than referring, provided its own criteria are met.

The medicine

Name, form and strength	Dexamethasone 0.1% w/w, neomycin sulfate 0.5% w/w and acetic acid 2% w/w ear spray.
Legal category	POM.
Dose and frequency	One metered spray into the affected ear three times daily, in accordance with the current SPC. Prime the pump before first use.
Quantity to be supplied	One bottle. No repeat supply under this PGD.
Maximum treatment period	7 days. One course per episode. A patient not improving at the end of the course is referred, not re-supplied.
Route and method of administration	Topical spray into the external auditory canal. Shake well before use. Keep the head tilted, or lie on the side, for a few minutes after applying.
Storage	Store below 25 degrees Celsius. Do not refrigerate or freeze. Store upright.
Drug interactions	Do not use other ear preparations at the same time. Refer to the current SPC.
Adverse effects	Local irritation, stinging or burning on application, pruritus, and hypersensitivity reactions including contact sensitisation to neomycin. Visual disturbance has been reported with corticosteroid use.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given, and from whom. Where the patient is a child, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. ● Patient name, address, date of birth, and the GP with whom they are registered. ● **THE OTOSCOPY FINDING, in terms: which ear, the state of the canal, and that the tympanic membrane was seen and was intact. Where the drum could not be visualised, that fact and what was done about it.** ● **That every red flag in Appendix 1 was asked about or looked for, and was absent.** ● **Diabetes and immunosuppression status, and the severity of pain.** ● How long symptoms have been present, and any treatment already tried. ● **Any previous episode in the last 12 months, and any previous course supplied under this PGD.** ● Which product was supplied and why. ● Name, form, strength, dose, quantity and date of supply. ● Batch number and expiry date of the product supplied. ● Name and registration number of the healthcare professional supplying. ● Advice given, including advice given if the patient is excluded or declines. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD.

	Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused product to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	● Shake the bottle well. One spray into the sore ear, three times a day, for 7 days. ● Keep your head tilted or lie on your side for a few minutes afterwards. ● **Keep water out of the ear for the whole course and a week after. No swimming.** ● Nothing goes in the ear: no cotton buds, no earplugs. ● **Stop and tell us if the ear becomes more irritated or you get a rash. Some people become allergic to neomycin.**
Follow-up and when to get help urgently	**Seek help the SAME DAY for severe unrelenting pain (especially with diabetes or a weakened immune system), any facial weakness or drooping, swelling or pain in the bone behind the ear, or a temperature.** If you are no better after 7 days, go to your GP. Do not start a second course.

Appendix 1: Red flags. Ask and look for these before supplying anything

Any one of these excludes. The first three are emergencies.

Severe unremitting pain in a patient with diabetes or immunosuppression	• Suspect necrotising (malignant) otitis externa. • EMERGENCY ENT REFERRAL the same day. Needs imaging and intravenous antibiotics. • Most common in older patients with diabetes. Pain out of proportion to the appearance of the canal is the clue.
Facial nerve palsy	• Weakness or drooping on one side of the face. • EMERGENCY ENT REFERRAL the same day.
Mastoid signs	• Pain, swelling, redness or tenderness behind the ear, or a pinna pushed forward and down. • Suspect mastoiditis. EMERGENCY REFERRAL the same day.
Systemic illness	• Fever, rigors, or the patient looks unwell. • Refer the same day.
Spreading infection	• Cellulitis of the pinna, face or neck. • Refer the same day.
Failed treatment	• No improvement after 2 weeks, or after a full course under this PGD. • Refer. Do not supply a second course.
Fungal debris	• White or black fuzzy debris in the canal. • Antibacterial treatment will not help and may worsen it. Refer.
Foreign body	• Refer for removal.
Drum not visualised, perforated, or grommet in situ	• Do not supply the neomycin spray under any circumstances. • Refer.
Recurrent or chronic	• 3 or more episodes in 12 months, or symptoms over 3 months. • Refer to ENT for investigation.

Appendix 2: Key references

- NICE Clinical Knowledge Summary: Otitis externa.
- Summary of Product Characteristics for ciprofloxacin 2mg/ml ear drops solution in single-dose container, and for dexamethasone with neomycin sulfate and acetic acid ear spray, current versions.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Authorisation

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

GET REAL HEALTH

Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	8 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● Ciprofloxacin 2mg/ml single-dose ear drops added as a second, and preferred, arm. Version 001 authorised only the dexamethasone, neomycin and acetic acid spray, while the consultation tool for this service recommends ciprofloxacin drops. The document now authorises the medicine that is actually being supplied. ● Ciprofloxacin drops are preferred where the tympanic membrane cannot be clearly seen, in pregnancy and while breastfeeding, and in children aged 1 to under 2, none of which the spray covers. ● Otoscopy is now a required step and a required, separately signed-off competency. The tympanic membrane must be visualised and recorded as intact before either product is supplied. ● The red flags that previously appeared only in the guidance summary are now exclusion criteria in both arms, in Appendix 1. ● Severe unremitting pain in a patient with diabetes or immunosuppression is an explicit exclusion and an emergency ENT referral, being the presentation in which necrotising otitis externa is missed. ● Mastoiditis added as a red flag and exclusion. It was not mentioned anywhere in version 001. ● Fungal debris, foreign body, spreading infection and new neurological symptoms added as exclusions. ● One course per episode, with a referral trigger rather than a repeat. Recurrent otitis externa is defined as 3 or more episodes in 12 months and is referred to ENT. ● Records must now carry the otoscopy finding in terms, the red flag check, diabetes and immunosuppression status, and any previous episode in the last 12 months. ● Aural toilet clarified as a referral, not something to attempt in the pharmacy.

Previous versions

001	Development and issue of new PGD. Dexamethasone, neomycin sulfate and acetic acid ear spray only, ages 2 and over.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
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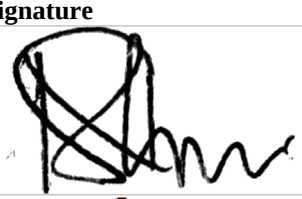
Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Ciprofloxacin arm age boundary stated in months: 12 months or over included, under 12 months excluded, matching the licence, the cover and the spray arm; the whole-year wording admitted and excluded a child aged 1 at the same time.
Spray maximum treatment period fixed at 7 days, one course per episode; the 14 day extension contradicted the one bottle, no repeat supply and one course per episode rules.
Spray counselling states the 7 day course length.
Batch number and expiry date added to the records of both arms.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 9 September 2026

Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.